



NAME : Mr.GURRAM SREEVARDHAN REDDY	UHID NO/Visit ID : KAD196.00152179/KAD196.198876
Age/Gender : 24 Y 1 M 24 D/M	Collected : 10/Sep/2026 07:54AM
Ref Doctor : SELF	Received : 10/Sep/2026 11:08AM
Ref. Cust : PHLB_1912713	Reported : 10/Sep/2026 11:27AM
Client Code : KAD196	Barcode : MB2353317

MEDIBUDDY SCREENING HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Range	Method
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COMPLETE BLOOD COUNT (CBC) - 25 TESTS

Haemoglobin	16.3	g/dL	13 - 17	Spectrophotometry - (non cyanide)
Total WBC Count	5830	Cells/cumm	4000 - 10000	Electrical Impedance
RBC Count	5.34	Millions/cumm	4.5 - 5.9	Electrical Impedance
Platelet Count	292	10 ³ /μL	150 - 450	Electrical Impedance
Packed Cell Volume (PCV)	47.7	%	40 - 50	Calculated
Mean Corpuscular Volume (MCV)	89.3	fL	83 - 101	Calculated
Mean Corpuscular Hb. (MCH)	30.5	pg	27 - 32	Calculated
MCHC	34.2	g/dL	31.5 - 34.5	Calculated
MPV	9.2	fL	7 - 11	Calculated
Platelet Crit	0.269	%	0.15 - 0.62	Calculated
RDW CV	13.5	%	11.6 - 14	Calculated
RDW-SD	43.00	fL	39.5 - 46.0	Calculated
PDW	16.2	fL	11 - 22	Calculated
P-LCR	21.0	%	18 - 50	Calculated

Differential Count by Flowcytometry/Microscopy

Neutrophils	57.7	%	40 - 80	Impedance/microscopy
Lymphocytes	33.6	%	20 - 40	Impedance/microscopy
Eosinophils	2.6	%	1 - 6	Impedance/microscopy
Monocytes	5.6	%	2 - 10	Impedance/microscopy
Basophils	0.5	%	0 - 2	Impedance/microscopy
Absolute Neutrophil Count	3363.91	cells/cumm	2000 - 7000.	Calculated
Absolute Basophils Count	29.15	Cells/cumm	0 - 100	Calculated
Absolute Lymphocyte Count	1958.88	Cells/cumm	1000 - 3000	Calculated
Absolute Eosinophil Count	151.58	Cells/cumm	50 - 500	Calculated
Absolute Monocyte Count	326.48	Cells/cumm	200 - 1000	Calculated

Smear				
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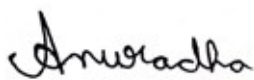
RBC's : Normocytic normochromic in nature.

WBC's: Total counts within normal range. Differential cells study shows normal morphology and distribution.

Platelet's: Adequate in number with normal morphology.

No haemoparasites seen.

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Consultant Pathologist



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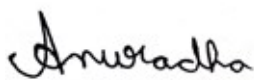
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IMP: Normocytic Normochromic Blood Picture.

Interpretation Notes:-

A complete blood count gives information regarding the cell types in person's blood and concentration of haemoglobin. Cells that circulate in the blood are generally divided into three types: RBC, WBC and platelets. Abnormally high or low counts may occur in physiological conditions and in diseased states and requires clinical correlation. Differential counts and RBC indices help in further understanding of the likely aetiology.

NOTE: This report has been generated by a fully automated analyser after counting thousands of cells and hence differential count may appear as decimalized numbers.


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MEDIBUDDY SCREENING HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Range	Method
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GLUCOSE - FASTING (FBS) , NAF PLASMA

Fasting Glucose	85.60	mg/dL	70 - 110	Hexokinase
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Ref.for Biological Reference Intervals: American Diabetic Association.

Interpretation Notes:-

A blood glucose test measures the glucose levels in your blood. Glucose is a type of sugar. It is your body's main source of energy. Symptoms of high blood glucose levels include increased thirst, more frequent urination, Blurred vision, Fatigue, Wounds that are slow to heal. Symptoms of low blood glucose levels include Anxiety, Sweating, Trembling, Hunger and Confusion. Blood glucose test is required to check certain risk factors for diabetes. These include Being overweight, Lack of exercise, Family member with diabetes, High blood pressure, Heart disease.

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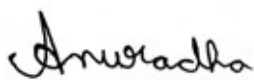
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LIPID PROFILE , SERUM

Total Cholesterol	167.00	mg/dL	Desirable : < 200 Borderline High : 200 - 239 High : ≥ 240	Enzymatic colorimetric
HDL Cholesterol	34.30	mg/dL	Low: < 40, High: > 60	Enzymatic colorimetric
Total Triglycerides	164.00	mg/dL	Desirable Level : 150 Borderline : 150-199 High : 200-499 Very High : 500	Enzymatic colorimetric
VLDL Cholesterol	32.80	mg/dL	≤ 30	Calculated
LDL Cholesterol	99.90	mg/dL	Optimal: < 100 Above Optimal: 100 - 129 Borderline: 130 - 159 High: 160 - 189 Very High: ≥ 190	Calculation
Non - HDL Cholesterol	132.70	mg/dL	≤ 130	Calculated
Chol / HDL Ratio	4.87		Low Risk: 3.3 - 4.4 Average Risk: 4.5 - 7.1 Moderate Risk: 7.2 - 11.0 High Risk : > 11.0	Calculated
TGL / HDL Ratio	4.8		Optimal: < 2.5 Mild to moderate risk: 2.5 - 5.0 High Risk: > 5.0	Calculated
HDL / LDL Ratio	0.34			Calculated
LDL / HDL Ratio	2.91		Desirable: 0.5 - 3.0 Borderline Risk: 3.0 - 6.0 High Risk: > 6.0	Calculated

Interpretation Notes:-

Lipid profile measures the amount of cholesterol and fats called triglycerides in the blood. These measurements give the doctor a quick snapshot of what's going on in blood. Cholesterol and triglycerides in the blood can clog arteries, making you more likely to develop heart disease.


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ASPARTATE AMINOTRANSFERASE (AST/SGOT) , SERUM

SGOT / AST	23.10	U/L	< 40.0	IFCC
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Interpretation Notes:-

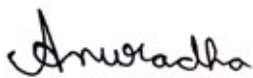
If AST levels are high, it may also be a sign of:

- chronic hepatitis
- damage from alcohol
- cholestasis, a decrease in bile flow
- heart, kidney, bone, or muscle damage
- liver cancers
- liver scarring, known as liver cirrhosis
- Very high AST levels are usually a sign of progressing liver damage, often due to acute hepatitis.

Low AST levels may indicate:

- Vitamin B6 deficiency
- kidney disease
- liver disease
- cirrhosis
- cancer
- autoimmune conditions
- genetic conditions

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MEDIBUDDY SCREENING HEALTH CHECK

Test Name	Result	Unit	Bio. Ref. Range	Method
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ALANINE AMINOTRANSFERASE - (ALT / SGPT) , SERUM

SGPT / ALT	27.10	U/L	< 42.0	IFCC
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Interpretation Notes:-

High levels of ALT can be a result of damage or injury to cells. Because ALT is most concentrated in the liver, abnormal ALT test results are generally associated with conditions affecting the liver, such as inflammation (hepatitis) and scarring (cirrhosis).

At the same time, ALT can be elevated without any underlying health problem. Multiple factors can affect ALT; in most cases, high ALT is not a sign of severe liver disease.

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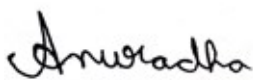
Test Name	Result	Unit	Bio. Ref. Range	Method
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KIDNEY FUNCTION TEST (BASIC) - 6 TESTS

Urea	17.51	mg/dL	16.6 - 48.5	Urease
BUN	8.18	mg/dL	7 - 18	Calculated
Creatinine	0.95	mg/dL	Males: 0.70 - 1.20	Modified Jaffe Kinetic
Urea/Creatinine Ratio	18.35			
BUN/Cr Ratio	8.57			Calculated
Uric Acid	5.95	mg/dL	Male: 3.4 - 7.0 Female: 2.4 - 5.7	Uricase

Interpretation Notes:-

Kidney/Renal function tests (KFT/RFT) are usually ordered when a patient has risk factors for kidney dysfunction such as hypertension, diabetes, cardiovascular disease, obesity, elevated cholesterol or a family history of kidney disease. It may also be ordered when someone has signs and symptoms of kidney disease, though in early stage often no noticeable symptoms are observed. Kidney panel is useful for general health screening; screening patients at risk of developing kidney disease; management of patients with known kidney disease.


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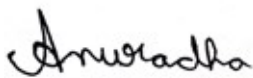
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THYROID STIMULATING HORMONE (TSH) , SERUM

Thyroid Stimulating Hormone (TSH)	2.43	μIU/mL	0.27 - 4.2	ECLIA
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Interpretation Notes:-

TSH interacts with specific cell receptors on the thyroid cell surface and exerts two main actions. The first action is to stimulate cell reproduction and hypertrophy. Secondly, TSH stimulates the thyroid gland to synthesize and secrete T3 and T4. The ability to quantitate circulating levels of TSH is important in evaluating thyroid function. It is especially useful in the differential diagnosis of primary (thyroid) from secondary (pituitary) and tertiary (hypothalamus) hypothyroidism. In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hypothyroidism, TSH levels are low. TRH stimulation differentiates secondary and tertiary hypothyroidism by observing the change in patient TSH levels. Typically, the TSH response to TRH stimulation is absent in cases of secondary hypothyroidism, and normal to exaggerated in tertiary hypothyroidism. Historically, TRH stimulation has been used to confirm primary hyperthyroidism, indicated by elevated T3 and T4 levels and low or undetectable TSH levels. TSH assays with increased sensitivity and specificity provide a primary diagnostic tool to differentiate hyperthyroid from euthyroid patients.


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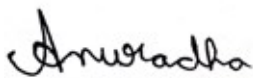
Test Name	Result	Unit	Bio. Ref. Range	Method
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VITAMIN - B12 , SERUM

Vitamin - B12	115	pg/mL	>18 Years : 191 - 663	ECLIA
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Interpretation Notes:-

Vitamin B12 and folate are critical to normal DNA synthesis, which in turn affects erythrocyte maturation. Vitamin B12 is also necessary for myelin sheath formation and maintenance. The body uses its B12 stores very economically, reabsorbing vitamin B12 from the ileum and returning it to the liver so that very little is excreted. Clinical and laboratory findings for B12 deficiency include neurological abnormalities, decreased serum B12 levels, and increased excretion of methylmalonic acid. The impaired DNA synthesis associated with vitamin B12 deficiency causes macrocytic anemias. These anemias are characterized by abnormal maturation of erythrocyte precursors in the bone marrow, which results in the presence of megaloblasts and in decreased erythrocyte survival. Pernicious anemia is a macrocytic anemia caused by vitamin B12 deficiency that is due to lack of intrinsic factor. Low vitamin B12 intake, gastrectomy, diseases of the small intestine, malabsorption, and trans-cobalamin deficiency can also cause vitamin B12 deficiency.


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EGFR (ESTIMATED - GLOMERULAR FILTRATION RATE) WITH SERUM CREATININE , SERUM

Creatinine	0.95	mg/dL	Males: 0.70 - 1.20	Modified Jaffe Kinetic
eGFR - Glomerular Filtration Rate	113.89	mL/min/1.73m ²	Refer Below	Calculated by using CKD EPI 2021 Formula

Biological Reference Ranges (eGFR)

Normal : >90 mL/min/1.73 m²Mildly Decreased : 60-90 mL/min/1.73 m²Mildly to Moderately Decreased: 45-59 mL/min/1.73 m²Moderately to Severely Decreased: 30-44 mL/min/1.73m²Severely Decreased: 15-29 mL/min/1.73m²Kidney Failure: <15 mL/min/1.7m²

A normal eGFR for adults is greater than 90 mL/min/1.73m², according to the National Kidney Foundation. (Because the calculation works best for estimating reduced kidney function, actual numbers are only reported once values are less than 60 mL/min/1.73m²).

An eGFR below 60 mL/min/1.73m² suggests that some kidney damage has occurred. The test may be repeated to see if the abnormal result persists. Chronic kidney disease is diagnosed when a person has an eGFR less than 60 mL/min/1.73m² for more than three months.

A person may have some kidney damage even with an eGFR greater than 90 mL/min/1.73m². Other evidence, such as increased urine albumin, may indicate some degree of kidney damage. Thus, a person's eGFR should be interpreted in relation to the person's clinical history and present conditions.

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*** End Of Report ***

Disclaimer:

1. It is presumed that the specimen belongs to the patient named or identified in the test request form. 2. Results pertain to the specimen submitted and all results are dependant on the quality of sample received in the laboratory. 3. The test results are for information and interpretation of the referring doctor or other medical professional who understands reporting units, reference ranges and limitations of terminologies. 4. Laboratory investigations help in arriving at a diagnosis and should be clinically correlated. 5. This report is not valid for medico-legal purposes. 6. Partial reproduction of this report is not permitted/not valid.

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